

CHAROTAR UNIVERSITY OF SCIENCE AND TECHNOLOGY

University Examination

Date: 28/11/2018

Day: Wednesday

Time: 10:00 am to 10:30 am

Total Marks: 20

Student's ID No.									
Student's Name: _____									
Student's Sign : _____									
Supervisor's Name : _____									
Supervisor's Sign: _____									

Answer Sheet for Multiple Choice Questions (MCQs).	
Examination : MPT- November 2018	
Semester: [4]	
Section : I	
Course Code : PT916	
Course Name: Paediatric Neuro Physiotherapy	

Important Instructions:

- There are two sections in the paper. **SECTION –I** contains 20 Questions and **SECTION – II** contains 3 Questions.
- **SECTION –I** should be answered in the question paper itself & **SECTION – II** should be answered in separate answer book.
- Marks on the right side suggest the total marks of that question.
- Maximum time allotted for **SECTION – I** is 30 minutes Question paper for **SECTION – II** will be given once the **SECTION – I** is completed within the stipulated time and returned. However, Candidates who complete **SECTION – I** earlier than 30 minutes can collect main question paper for **SECTION –II** immediately after handing over the **SECTION-I** question paper with answers.
- **SECTION – I** should be attached with the main answer book at the end of the examination and should be submitted to the supervisor.

SECTION-I

Multiple Choice Questions: (MCQs)
(Encircle the correct answer)

(20 × 1 = 20)

1. Velly's sign is usually seen in _____
 - a. Limb girdle muscular dystrophy
 - b. Backer's muscular dystrophy
 - c. Duchenne muscular dystrophy
 - d. Myotonic muscular dystrophy
2. Which of the following statement is **not true** with regard to upper brachial plexus injury
 - a. Most common type of brachial plexus injury
 - b. Commonly result due to an obstetric injury
 - c. Sensory involvement of the ulnar side of the hand
 - d. Weakness of deltoid, biceps and wrist extensors

(P.T.O)

3. Downward traction injury of the brachial plexus predominantly affects
 - a. C7, 8, T1
 - b. C8, T1
 - c. C5, 6, 7
 - d. C5, 6
4. Which one of the following is the most frequently occurring form of spinal dysraphism?
 - a. Spina bifida occulta
 - b. Spina bifida cystica and meningocele
 - c. Spina bifida cystica and Meningomyelocele
 - d. Myeloschisis
5. APGAR score is used to assess all system Except:
 - a. Musculoskeletal System
 - b. Nervous System
 - c. Cardiac system
 - d. Respiratory system
6. Persistence of this reflex affects the prognosis of a child.
 - a. Righting reactions
 - b. Flexion withdrawal
 - c. Parachute reflex
 - d. Abdominal reflex
7. A patient reports to physical therapy with the diagnosis of a lesion in the lateral cord of the brachial plexus. Which of the following would you most likely detect upon the assessment of this patient?
 - a. Paralysis of the biceps and coracobrachialis
 - b. Paralysis of the deltoid
 - c. Paralysis of wrist extension
 - d. Paralysis of the hand
8. What would you consider as the primary goal of child centered intervention when you are planning the program of a 7 year old child with spastic diplegic Level III CP with a sound cognitive function?
 - a. Prescribe orthosis and mobility aids to optimize function
 - b. Optimize his school participation and peer interaction
 - c. Improve his gait and posture
 - d. Increase his strength and endurance
9. The most common type of cerebral palsy is _____
 - a. Spastic
 - b. Athetoid
 - c. Ataxic
 - d. Mixed
10. The following tools measure the motor function in children except _____
 - a. Gross motor function measure
 - b. Gesell developmental schedules
 - c. Alberta infant motor scale
 - d. Movement assessment of Infants

11. Children with cerebral palsy most often develop tightness and contracture of the following group of muscle except _____
- Ankle plantar flexors, Knee extensors and Hip flexors
 - Knee extensors, hip adductors and hip flexors
 - Hip adductors, Knee flexors and ankle dorsi flexors
 - Hip flexors, knee flexors and ankle plantar flexors
12. Which of the following is not a sign and symptom of shunt malfunction in toddlers?
- Vomiting
 - Irritability
 - Bulging fontanelle
 - Eczema, redness along the shunt tract
13. You are receiving a phone call in a pediatric physiotherapy clinic. Which of the following reports most requires your immediate attention for care or referral?
- A 8 year-old boy has been vomiting and appears to have slower movements and has a history of an atrio-ventricular shunt placement.
 - A 10 year-old girl feels a dull pain in her abdomen after doing sit-ups in gym class.
 - A 7 year-old boy has been having a low fever and headache for the past 3 days that has history of an anterior knee wound.
 - A 7 year-old girl that had a cast on her right ankle is complaining of itching.
14. Shared decision making within Patient and Family Centered Care looks like the following:
- The doctor sharing the risks and benefits of a procedure to his/her client
 - The physiotherapist formulating the entire treatment plan for their patient
 - The Patient selecting a treatment option that satisfies their personal beliefs
 - A and C
15. The following deformities are commonly seen in patients with Spina Bifida except _____
- Pes calcaneus, calcaneovarus and calcaneovalgus
 - Telipes equinovarus
 - Pes anserinus and pes planus
 - Pes cavus and claw toe
16. Which of the following is the least important criteria in choosing a specific device or adaptive equipment for children with disability?
- Dimension of the apparatus
 - Cost of the apparatus
 - Reputation of the manufacturer
 - Availability of optional adaptations
17. Making a good first impression is an important part of patient-and family centered care because?
- Making a good first impression decreases the likelihood of legal proceedings
 - It saves time for caregivers
 - Treating patients with respect begins the moment that you first meet them
 - It will help in satisfying the needs of the family

(P.T.O)

18. The purpose of Patient and Family Centered Care is to:
- a. To ensure patient safety and attain the best desired outcomes
 - b. To ensure that Healthcare providers retain their patients
 - c. To help law makers create reform in healthcare
 - d. To allow patients to make an independent decision on the type of care they wish to receive.
19. Newborn assessment at the first 1 minute using the APGAR score reveals that the newborn is bluish in colour, HR is <100, has some flexion in the limbs but actively crying. What do you think the APGAR score is?
- a. 3 or 4
 - b. 6 or 7
 - c. 8 or 8
 - d. 5 or 6
20. What is the GMFCS level of a child with CP, who walks with assistive mobility devices, may be able to climb stairs using rails and propel a manual wheelchair and need assistance for long distances or uneven surfaces.
- a. Level I
 - b. Level II
 - c. Level III
 - d. Level IV

CHAROTAR UNIVERSITY OF SCIENCE & TECHNOLOGY

4th Semester of M.P.T. Examination

November 2018

PT916 Paediatric Neurophysiotherapy

Date: 28/11/2018

Time: 10:30 am to 1:00 pm

Maximum Marks: 50

Instructions:

1. There are two Sections in the paper. **SECTION –I** contains 20 Questions and **SECTION – II** contains 3 Questions.
2. **SECTION –I** should be answered in the question paper itself & **SECTION – II** should be answered in separate answer book.
3. Marks on the right side suggest the total marks of that question.
4. Maximum time allotted for **SECTION – II** is 2 Hr 30 min.
5. Draw the figure where necessary.

SECTION-II

Q. 1. Short answers (5 out of 7) (Answer not exceeding 50 words)

(5 × 2 = 10)

- a. Mini core disease
- b. Myotonia congenita
- c. Orthotic prescription in cerebral palsy
- d. Spina bifida cystica.
- e. Antenatal causes of risk babies.
- f. Motor indicators of the model of determinants of change for children with CP.
- g. Limb girdle muscular dystrophy

Q. 2. Short Notes (4 out of 5) (Answer not exceeding 150 words)

(4 × 5 = 20)

- a. Family needs of parents of children and youth with cerebral palsy
- b. Explain the differences between family centered care and child centered care of rehabilitation.
- c. Explain any two primitive reflexes which are of importance while evaluating a newborn with birth dystocia suspecting of BPI.
- d. Constraint induced movement therapy in brachial plexus injury.
- e. Approach to sharing unbiased and complete information with parents about their child with Duchene muscular dystrophy.

Q. 3. Essay (2 out of 3)

(2 × 10 = 20)

- a. An 8 years old male child is having difficulty in rising from the floor, has a slow speed of walking and needs to take frequent rest in between. On examination bilateral trendlenburg is observed on stance with 40⁰ hip flexion contracture with IT

tightness and 25° plantar flexion contracture. MMT is 3+/5 of the gross lower limb. DTR are absent. Identify the condition, write down the assessment and discuss the management for the same. (1+4+6)

- b. A three year old child had an Obstetric Brachial Plexus Injury of the right side. He is functional in his basic activities. He has been classified as Grade III according to Modified Mallet Classification of Brachial Plexus injury. Explain various ways to increase the right upper limb function in the form of the exercises, play activities and recreational activities.
- c. An eleven year old boy has spastic triplegic CP, who can independently sit and perform basic bed mobility but dependent in transfer activity and self-care. His school participation is limited, as he needs special care in transfer and seating arrangements. His cognitive function is sound. Using the ICF framework of clinical decision-making, plan a service intervention to optimize his participation in life and improve his quality of life.